

DPR: Introduction to Behavioral Medicine: The Components of the Patient Encounter Principles and Practices of Osteopathic Medicine 1

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Session Objectives

THE STUDENT PHYSICIANS WILL BE ABLE TO:

1. **Demonstrate** the components of the **history and physical** examination of the patient with a behavioral or mental health complaint
2. **Evaluate** the patient utilizing **history taking, observational skills, active listening, facilitated responses in addition to the physical exam**
3. **Distinguish normal versus abnormal encounter** findings-use of screening measures
4. **Anticipate** and discuss common **abnormal** encounter findings
5. Recall the appropriate way to **document** common normal and abnormal findings of history and examination intake by **writing a SOAP note within 9 minutes**.

MAIN POINTS:

- History considerations for the patient with a behavioral or mental health complaint
- Physical exam
- Osteopathic Treatments
- SOAP note writing



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Epidemiology & Impact

Prevalence of Adult Mental Health:

- **1 in 5 adults in America experience a mental illness. In 2019, just prior to the COVID-19 pandemic, 19.86% of adults experienced a mental illness, equivalent to nearly 50 million Americans.**
- **More than 50% of adults with mental health concerns are receiving no treatment whatsoever.**
- **Almost 5% of adults report experiencing suicidal ideation.**
 - **<https://www.nssbehavioralhealth.com/nss-blog-the-state-of-mental-health-in-america-2022-adult-prevalence-and-access-to-care>**

*Mental Health Statistics in the US 2025 | Key Facts - The World Data

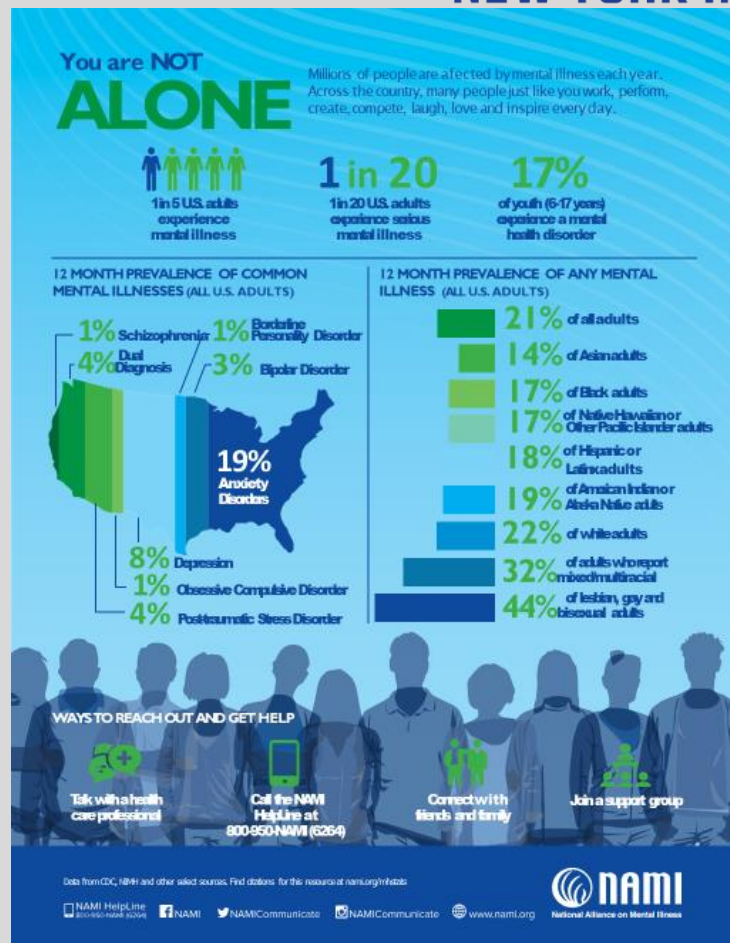
- <https://www.nami.org/NAMI/media/NAMI-Media/Infographics/MulticulturalMHFacts10-23-15.pdf>

Statistics	Details	Mental Health Facts in the US 2025
59.3 million	23.1% of all US adults in 2022	Total Adults with Mental Illness
15.4 million	6.0% of all US adults	Adults with Serious Mental Illness
49.5%	Nearly half of all teens aged 13-18	Adolescents with Mental Disorders
36.2%	Ages 18-25 have highest rates	Young Adults Most Affected
26.4% females vs 19.7% males	Women 6.7% more likely to have mental illness	Gender Disparity
50.6%	Only half receive mental health treatment	Treatment Gap
NIMH (2024), NSDUH 2022; compiled by The World Data (2025).		

Prevalence of Adult Mental Health

by Race

https://nami.org/NAMI/media/NAMI-Media/Infographics/NAMI_YouAreNotAlone_2020_FINAL.pdf



Epidemiology – Adults

- **Nearly 1 in 25 (10 million) adults in America live with mental illness**
 - **18.1% (42 million) of adults live with anxiety disorders**
 - **6.9% (16 million) of adults live with major depression**
 - **2.6% (6.1 million) of adults live with bipolar disorder**
- **Approximately 10.2 million adults have co-occurring mental health and addiction disorders**
- **More than 7% of Americans report a substance use disorder**
- ***Depression is the **leading** cause of disability worldwide, and is a major contributor to the global burden of disease**

<https://www.nami.org/NAMI/media/NAMI-Media/Infographics/GeneralMHFacts.pdf>

Epidemiology - Adults

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- Only **41% of adults** in the U.S. with a mental health condition **received mental health services** in the past year
- Approximately 24% of state prisoners have “a recent history of a mental health condition”
 - **Approximately 26% of homeless adults staying in shelters live with serious mental illness**
- Adults in the U.S. living with serious mental illness die on average 25 years earlier than others
 - Largely due to treatable medical conditions that go untreated
- **90% of those who die by suicide have an underlying mental illness.**
 - Suicide is the 10th leading cause of death in the U.S.
 - <https://www.nami.org/NAMI/media/NAMI-Media/Infographics/GeneralMHFacts.pdf>

Suicide Statistics/Basic Facts-U.S.

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***Suicide is the most preventable cause of death**

- 8th leading cause of death
 - 30,000-40,000 people die by suicide per year
 - 5 million living Americans have made an attempt
 - 1 person every 18 minutes commits suicide
 - U.S ranked #27 in the world of highest suicide rate
 - 2nd or 3rd leading cause of death in youth (tripled since 1950s)
 - #1 Risk Factor: Untreated Depression (2 out of 5 don't get treatment)
-
- **JULY 2022-988-National Emergency Line for MH**

* QPR Institute, Paul Quinnett, PhD.

- **Epidemiology of – youth/adolescents**

- 1 in 5 children ages 13-18 have, or will have a serious mental illness
 - 20% have live with a mental health condition
 - 11% have a mood disorder
 - 8% have an anxiety disorder
- Suicide is the **3rd leading cause of death in ages 10 - 24**
- 50% of all lifetime cases of mental illness begin by age 14 and 75% by age 24
- 37% of students with a mental health condition age 14 and older drop out of school—**the highest dropout rate of any disability group**
- 70% of youth in state and local juvenile justice systems have a mental illness

<https://www.nami.org/NAMI/media/NAMI-Media/Infographics/Children-MH-Facts-NAMI.pdf>

Disparities

- There are racial and ethnic disparities
 - Lower rates of health insurance
 - Less access to treatment
 - 11.1% of Americans with a mental illness are uninsured.
 - Poorer quality of care
 - Encounter racism, bias, discrimination in treatment settings
 - Language barriers
 - Higher levels of stigma
- <https://www.nami.org/NAMI/media/NAMI-Media/Infographics/MulticulturalMHFacts10-23-15.pdf>

Epidemiology

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- LGBTQ individuals are **2 or more times** more likely as straight individuals to have a mental health condition.
 - 11% of transgender individuals reported being ***denied care*** by mental health clinics due to **bias or discrimination**.
 - LGBTQ youth are 2 to 3 times more likely to attempt suicide than straight youth.
- 70% of LGBTQ youth stated that their mental health was "poor" most of the time or always during COVID-19.
- 48% of LGBTQ youth reported they wanted counseling from a mental health professional but were unable to receive it in the past year

[*2024 U.S. National Survey on the Mental Health of LGBTQ+ Young People in New York | The Trevor Project](#)

- <https://www.thetrevorproject.org/survey-2021/>
 - <https://www.nami.org/NAMI/media/NAMI-Media/Infographics/MulticulturalMHFacts10-23-15.pdf>

Impact of Mental Health Disorders

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- Increases risk of
 - Suicide
 - Addiction disorder
 - Incarceration
 - Homelessness
 - Joblessness
 - Having chronic medical conditions
 - Dropping out of high school
- Costs **\$193.2 billion** in lost earning every year in the US

- <https://www.nami.org/Learn-More/Mental-Health-By-the-Numbers>

The Evaluation of an Adult Patient with a Behavioral Complaint

Basic format

- **Introduction & set the agenda**
- **Chief complaint & current history**
- **Review of Systems**
- **Past psychiatric history**
- **Medical history**
- **Personal history**
- **Social history**
- **Physical examination (and OMM considerations)**
- **Summary & Closing**

Introduction & set the agenda

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- Set up safe environment for you and the patient
 - Introduce yourself and explain why you are seeing the patient
- Acknowledge that what you will be discussing is often difficult
 - *Assure the patient of confidentiality*
- Gather basic demographic information from the patient such as name, age, date of birth

During the interview

- Maintain professionalism at all times
 - Be aware of pre-existing biases in yourself-non-judgmental
- Use the patient's own language
 - Repeating their words in your statements can help show the patient you are listening and trying to understand.
- Use appropriate **transition statements** when you move onto topics that are sensitive or personal
- Educate/counsel patient **after** you have gathered the history & completed the physical exam

Observation of the patient

How do the **patient's actions/appearance** contribute to the information they are giving you?

While interviewing the patient, you will concomitantly observe the patient's

- Appearance
 - Speech patterns-pitch, tone
 - Behavior
 - Attitude
 - Mental status
 - Posture
 - Mood
 - Affect
- Think back to the Cultural Competency and General appearance DPR lectures

Chief complaint & current history

- Chief complaint
 - In the patient's own words (always)
- HPI
 - Use open ended questions followed by closed ended questions for clarification
 - Obtain a thorough account and timeline of the patient's symptoms/concern
 - Patient Health Questionnaire-2 (PHQ-2) - Mental Disorders Screening-Depression
 - Generalized Anxiety Disorder Screening-2 (GAD-2)

Patient Health Questionnaire-2 (PHQ-2)

[Share](#)

The PHQ-2 inquires about the frequency of depressed mood and anhedonia over the past two weeks. The PHQ-2 includes the first two items of the PHQ-9.

- The purpose of the PHQ-2 is to screen for depression in a “first-step” approach.
- Patients who screen positive should be further evaluated with the PHQ-9 to determine whether they meet criteria for a depressive disorder.

Over the last 2 weeks , how often have you been bothered by the following problems?	Not at all	Several days	More than half the days	Nearly every day
1. Little interest or pleasure in doing things	☐ 0	☐ +1	☐ +2	☐ +3
2. Feeling down, depressed or hopeless	☐ 0	☐ +1	☐ +2	☐ +3

PHQ-2 score obtained by adding score for each question (total points)

Interpretation:

- A PHQ-2 score ranges from 0-6. The authors identified a score of 3 as the optimal cutpoint when using the PHQ-2 to screen for depression.
- If the score is 3 or greater, major depressive disorder is likely.
- Patients who screen positive should be further evaluated with the [PHQ-9](#), other diagnostic instruments, or direct interview to determine whether they meet criteria for a depressive disorder.

PHQ-9

Patient Health Questionnaire-9 (PHQ-9)

Share

The PHQ-9 is a multipurpose instrument for screening, diagnosing, monitoring and measuring the severity of depression.

Over the **last 2 weeks**, how often have you been bothered by the following problems?

Not at all

Several days

More than half the days

Nearly every day

1. Little interest or pleasure in doing things

☐ 0	☐ +1	☐ +2	☐ +3
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2. Feeling down, depressed or hopeless

☐ 0	☐ +1	☐ +2	☐ +3
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3. Trouble falling asleep, staying asleep, or sleeping too much

☐ 0	☐ +1	☐ +2	☐ +3
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4. Feeling tired or having little energy

☐ 0	☐ +1	☐ +2	☐ +3
-------------------------	--------------------------	--------------------------	--------------------------

5. Poor appetite or overeating

☐ 0	☐ +1	☐ +2	☐ +3
-------------------------	--------------------------	--------------------------	--------------------------

6. Feeling bad about yourself - or that you're a failure or have let yourself or your family down

☐ 0	☐ +1	☐ +2	☐ +3
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7. Trouble concentrating on things, such as reading the newspaper or watching television

☐ 0	☐ +1	☐ +2	☐ +3
-------------------------	--------------------------	--------------------------	--------------------------

8. Moving or speaking so slowly that other people could have noticed. Or, the opposite - being so fidgety or restless that you have been moving around a lot more than usual

☐ 0	☐ +1	☐ +2	☐ +3
-------------------------	--------------------------	--------------------------	--------------------------

<https://www>

PHQ-9

9. Thoughts that you would be better off dead or of hurting yourself in some way

☐ 0	☐ +1	☐ +2	☐ +3
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PHQ-9 score obtained by adding score for each question (total points)

Interpretation:

- Total scores of 5, 10, 15, and 20 represent cutpoints for mild, moderate, moderately severe and severe depression, respectively.
- Note: Question 9 is a single screening question on suicide risk. A patient who answers yes to question 9 needs further assessment for suicide risk by an individual who is competent to assess this risk.

Interpretation

Provisional Diagnosis and Proposed Treatment Actions		
PHQ-9 Score	Depression Severity	Proposed Treatment Actions
0 – 4	None-minimal	None
5 – 9	Mild	Watchful waiting; repeat PHQ-9 at follow-up
10 – 14	Moderate	Treatment plan, considering counseling, follow-up and/or pharmacotherapy
15 – 19	Moderately Severe	Active treatment with pharmacotherapy and/or psychotherapy
20 – 27	Severe	Immediate initiation of pharmacotherapy and, if severe impairment or poor response to therapy, expedited referral to a mental health specialist for psychotherapy and/or collaborative management

<https://www.nyit.edu/college-of-osteopathic-medicine/>

Generalized Anxiety Disorder 2-item (GAD-2)

[Share](#)

The Generalized Anxiety Disorder 2-item (GAD-2) is a very brief and easy to perform initial screening tool for generalized anxiety disorder.¹

Over the **last 2 weeks**, how often have you been bothered by the following problems?

Not at all

Several days

More than half the days

Nearly every day

1. Feeling nervous, anxious or on edge

☐

0

☐

+1

☐

+2

☐

+3

2. Not being able to stop or control worrying

☐

0

☐

+1

☐

+2

☐

+3

GAD-2 score obtained by adding score for each question (total points)

Interpretation:

A score of 3 points is the preferred cut-off for identifying possible cases and in which further diagnostic evaluation for generalized anxiety disorder is warranted. Using a cut-off of 3 the GAD-2 has a sensitivity of 86% and specificity of 83% for diagnosis generalized anxiety disorder.

Generalized Anxiety Disorder 7-item (GAD-7)

Share

The Generalized Anxiety Disorder 7-item (GAD-7) is a easy to perform initial screening tool for generalized anxiety disorder¹.

Over the **last 2 weeks**, how often have you been bothered by the following problems?

Not at all

Several days

More than half the days

Nearly every day

1. Feeling nervous, anxious or on edge

☐

0

☐

+1

☐

+2

☐

+3

2. Not being able to stop or control worrying

☐

0

☐

+1

☐

+2

☐

+3

3. Worrying too much about different things

☐

0

☐

+1

☐

+2

☐

+3

4. Trouble relaxing

☐

0

☐

+1

☐

+2

☐

+3

5. Being so restless that it is hard to sit still

☐

0

☐

+1

☐

+2

☐

+3

6. Becoming easily annoyed or irritable

☐

0

☐

+1

☐

+2

☐

+3

7. Feeling afraid as if something awful might happen

☐

0

☐

+1

☐

+2

☐

+3

GAD-7 score obtained by adding score for each question (total points)

<https://www.nyit.edu/college-of-osteopathic-medicine/>

GAD-7 score obtained by adding score for each question (total points)

Interpretation:

When screening for anxiety disorders, a score of 8 or greater represents a reasonable cut-point for identifying probable cases of generalized anxiety disorder; further diagnostic assessment is warranted to determine the presence and type of anxiety disorder. Using a cut-off of 8 the GAD-7 has a sensitivity of 92% and specificity of 76% for diagnosis generalized anxiety disorder.^{2,3}

The following cut-offs correlate with level of anxiety severity:

- Score 0-4: Minimal Anxiety
- Score 5-9: Mild Anxiety
- Score 10-14: Moderate Anxiety
- Score greater than 15: Severe Anxiety

Based on a recent meta-analysis, some experts have recommended considering using a cut-off of 8 in order to optimize sensitivity without compromising specificity².

<https://www.hiv.uw.edu/page/mental-health-screening/phq-2>

Current history

- When did symptoms begin?
 - Acute versus gradual changes
 - What triggered the symptoms?
- How have the symptoms changed over time?
- Can they describe what they are feeling or experiencing?
- Anything make it better? Anything make it worse?
- How have they impacted them on daily basis at home, at work, at school, etc.?
- How have they been coping?

Current history

- **Assess suicide/homicide risk**
 - Have they had thoughts about hurting themselves or other?
 - Have they made any attempts at suicide?
 - If yes, do they have a specific plan or timeline? Do they have access to weapons or the items in their plan?

Specific questions about suicide

- Was there something that triggered it?
 - Was it planned or impulsive?
- Did they carry out their final acts like
 - Write a suicide note
 - Leave a last will & testament
 - End contracts with companies or work
- Did they take any precautions to prevent others from discovering them?

Suicide Questions

- What method of self-harm was involved?
- How were they discovered?
- How did they feel when help arrived?
- Do they still feel suicidal?

Self-harm

- The patient may be harming themselves in other ways
- Skin picking/scratching
- Hair pulling-BFRBs
 - Cutting
 - Burning
 - Abusing drugs or alcohol

Past psychiatric History

- Ask about other mental health disorders
 - Severity of illness
 - Treatments/Medications
- Ask about suicide or self-harm
- Ask about previous psychiatric hospitalizations

Medical history

- Past medical history
- Past surgical history
- Medications
- Allergies

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Family history

- Ask about history of mental health disorders, suicide and substance abuse
- Their role in the family
- Supportive environment
- Religious/Ethnic/Cultural Factors
- Relationships-platonic/intimate

Personal History

- Birth & early development
 - Pre-natal or post-natal complications
 - Any delays
- Childhood
 - Personality traits
 - Behavioral problems, School adjustment
- Adolescence
 - Personality traits
 - Behavioral problems, School adjustment

Social history

- Living situation
- Occupation
- Education
- Current or past drug use
- Current or past alcohol use-self-medicating?
- Current or past tobacco use
- Sexual history

Living situation

- Where do they live?
- Who do they live with?
 - Consider that if they have children, the children may be neglected or witnessing the patient's struggle & may need intervention
- Are they in a relationship?
 - What type of relationship? How is that relationship now?

Living situation

- Do they have a good support network?
- What is the quality of this network?
 - Can include: family, friends, neighbors, co-workers, teachers, religious or spiritual advisors or authorities, etc.
- Are they able to manage all of their activities of daily living independently?

Occupation

- What job does the patient have?
 - How are the relationships at work?
 - How are they performing at work?
- If they are not working, how are they coping financially? Are they in debt?
- Have they served in the military?
 - If so, what was their experience?

Education

- What is the highest degree attained?
- Academic successes
- Were there any concerns or problems while in school? Hx of ADHD/LD?

Social history

Current or past alcohol use:

- Do they drink alcohol?
- How much do they drink?
- Ask about the pattern (for example daily versus binge drinking)
- Use the CAGE questions if the person is drinking above recommended guidelines or you are concerned about abuse/dependence

CAGE Questionnaire for Detecting Alcoholism

Share

Questions

Points

C : Have you ever felt you should Cut down on your drinking?

☐ Yes +1

☐ No +0

A : Have people Annoyed you to by criticizing your drinking?

☐ Yes +1

☐ No +0

G : Have you ever felt Guilty about your drinking?

☐ Yes +1

☐ No +0

E : Have you ever had a drink first thing in the morning (Eye opener)?

☐ Yes +1

☐ No +0

CAGE score is obtained by adding the points (total points)

0 points

Social history

*Current or past drug use

- Do they use recreational drugs?
- What drugs do they use?-Gateway Drugs-marijuana
- How often do they use the drug(s)?
- How much do they use of the drug(s)?

Social history

- Current or past tobacco use
 - Do they use tobacco products?
 - Which tobacco product?
 - How much do they use on a daily basis?

Social history

- Sexual history
 - Ask about # of partners in the last 12 months,
 - Whether partners are men, women, or both
 - Use of barrier protection
 - Risky behavior/Impulsivity
 - For e.g., having sex with random partners they have just met

Review of Systems

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- Ask questions about main organ systems to help determine if there is a physiological component or cause to the psychiatric symptoms that the patient is feeling
 - Constitutional
 - Cardiovascular
 - Pulmonary
 - Endocrine
 - Gastrointestinal
 - Neurological

Physical examination

- May be dependent on the differential diagnosis
- Examine major organ systems to look for signs/symptoms of disease that might cause the patient's presenting complaint
 - Cardiovascular
 - Pulmonary
 - Endocrine
 - Gastrointestinal
 - Neurological

Mental status exam

- -Geriatric lecture
- You need to constantly observe as well as listen to the patient, in order to assess them properly
- There are multiple elements that you will pay attention to

Mental Status Exam

- State of consciousness-alert, awake
- Appearance
- General behavior
- Attitude toward the examiner
- Attention
- Orientation
- Psychomotor activity
- Mood
- Affect
- Speech and language-slurred, communication barriers
- Form of thought
- Content of thought
- Perceptions
- Judgement
- Memory
- Insight
- Intellectual functioning

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Mental Status Exam

State of Consciousness

- Alert, hyper-alert, lethargic, obtunded, etc.
- Appearance
 - Overall impression – clothes, grooming, apparent age, posture, apparent health, are they in distress, etc.
- General behavior
 - Mannerisms, gestures, repetitive behaviors, etc.
 - Eye contact – do they make appropriate eye contact or are they avoidant?

Mental Status Exam

- Attitude toward the examiner
 - Cooperative, hostile, defensive, seductive, evasive, ingratiating, etc.
- Attention
 - Concentration, digit or word recall, etc.
- Orientation
 - To person, place, time and situation/location

Mental Status Exam

*Psychomotor activity

- Increased, reduced, agitated, abnormal movement, etc.
- Mood-
 - How the patient say they feel (sustained emotion)
 - Stated mood versus observed mood
 - Overall emotional state (sad, happy, depressed, elated, anxious, irritable, etc.)

Mental status exam

Affect

- Outward expression of patient's inner emotions/experiences
- Full affect (full range of emotions) versus restricted, blunted or flat
- Appropriate versus inappropriate to the patient's thoughts

Mental status exam

Speech and language

- Rate (increased, pressured, slow, etc.)
- Tone (soft, angry, etc.)
- Volume
- Articulation-coherent?
- Language
 - aphasia
- Is it age/culturally appropriate?

Mental Status Exam

- Form of thought
 - Circumstantial, flight of ideas, tangential, evasive, loosening associations, perseverance, blocking
- Content of thought
 - Preoccupations, obsessions/compulsions, phobias, rituals, delusions, etc.
- Perceptions
 - Illusions, hallucinations, depersonalization, etc.

Mental Status Exam

- Judgement
 - The ability to come to a sensible conclusion on a specific matter when need to choose from multiple options
 - For example: an inebriated patient decides that they are going to drive home
 - Described as poor, fair, or good.
- Insight
 - Does the patient recognize that they are ill? Do they appropriately attribute their symptoms to their illness/impairment of functioning?
 - Described as intact or not-awareness

Mental Status Exam

- Memory
 - Immediate recall-what did eat for lunch?
 - Remote recall-who was President in 1992?
- Intellectual functioning-education
 - Fund of knowledge
 - Ability to discuss abstract concepts (like a proverb)
 - Ability to perform calculations

Differential diagnosis

- Dependent on all the elements of the history and examination
- Have to rule out **physiological disease/disorders** that could mimic or cause the patient's symptoms
 - For example: **hypothyroidism** in patient with depression
 - For example: **arrhythmia** in patient with anxiety-mimics anxiety

Diagnostic tests to consider

- Dependent on the differential diagnosis but typically include:
 - EKG
 - Complete Blood Count (CBC)-infection, anemic, B12 deficient
 - Comprehensive Metabolic Panel (CMP)-electrolytes, liver fx-effects cognition
 - Thyroid studies
 - Lipid panel
 - Urinalysis-UTI-altered mental status in Geriatrics
- These tests also help to establish a **baseline** since medications can have an effect on glucose, lipids, heart rhythms, etc.

Osteopathic Considerations

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- Assess areas of somatic complaints
- Perform an osteopathic structural exam
 - **Consider techniques to normalize parasympathetic and sympathetic-so body can get back to healing itself**
 - **Suboccipital Release**
 - **Rib raising**
- Treat appropriately based off your structural exam findings
 - Sometimes less is more-patient may not want this

Treatment Plan

- An explanation to the patient of the:
 - Differential diagnosis-most-least likely
 - Risks of untreated illness
 - Treatment options-education/counseling, additional tests, therapy, referrals, medications- As first year student you may not know this right now
 - Benefits and risks of treatment-educate about medications
- **Collaboration between the clinician and the patient about decisions pertinent to treatment**
 - Can define quantitative measures of symptoms, level of functioning, and quality of life to help measure progress

Treatment Plan

- Documentation should include:
 - Rationale for clinical tests-EKG-arrhythmia
 - Estimate of the patient's suicide/homicide risk
 - Including factors influencing risk
 - Rationale for treatment selection
 - Including discussion of the specific factors that influenced the treatment choice-why are you doing this?
 - Follow-up plans
 - Where and when at the very least-which office, weekly/biweekly

OUTPATIENT PSYCHIATRIC CLINIC

TEMPLATE MENTAL HEALTH PROGRESS NOTE:

SOAP is an acronym for the sections of a clinical note:

S = Subjective (The counselor describes his/her impressions of the client, and what the therapist understood the client to say)

Mr. A reported that he is sleeping 2-3 hours a night. He noted feeling sad, crying intermittently, and having suicidal ideation.

O = Objective (A factual account of what was observed)

Mr. A appeared lethargic. He rarely made eye-contact. His affect was flat, and he spoke only when queried.

A= Assessment (A synthesis of the Subjective and Objective portion of the note that includes diagnosis and clinical impressions)

Mr. A appears to be experiencing a major depressive episode.

P = Plan (The counselor describes the treatment plan that follows from the Assessment)

Mr. A will be referred for a medication evaluation. He has been put on suicide watch. He will be seen again tomorrow for further assessment

Summary & References

Summary Slide

- Mental health disorders have a strong societal and personal impact
- It is important that the history that is gathered from the patient is as thorough as possible and Individualized!
 - And Includes a proper mental status assessment as well as rule out other underlying or chronic conditions

References

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*Slides adapted from Dr. Joshi's DPR
Lecture

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- <https://www.nami.org/NAMI/media/NAMI-Media/Infographics/Children-MH-Facts-NAMI.pdf>

- Questions?
- Dr. Hoffman
- Lisa.Hoffman@nyit.edu
- Available for Virtual Office Hours

Lecture Feedback Form:

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